

Sources Sought Notice

Title: Community Health Workers/Representatives (CHW/CHR) Training
Agency: Indian Health Service
Issue Date: August 4, 2026
Due Date: August 11, 2026, no later than 5:00pm EST
Contacts: Responses must be submitted via email to cynthia.cason@ihs.gov and include "Community Health Workers – Your Company Name" in the Subject of the email.

This Sources Sought Notice (SSN) is for market research and planning purposes only and shall not be construed as a solicitation or an obligation or commitment to issue a solicitation by the Indian Health Service (IHS). This SSN seeks capability information from the types of entities listed below to assess their capabilities in fulfilling the requirements set forth and further detailed below. The information from this SSN will be used to determine the appropriate set-aside, if any, in accordance with applicable regulations. The Government encourages eligible entities to respond and identify their capabilities (to be assessed according to the following priority order):

1. Indian Small Business Economic Enterprises (ISBEE)
2. Indian Economic Enterprises (IEE) in accordance with HHSAR 326.603-1
3. Small business concerns in accordance with FAR 19.502-2 (b)(1) & (2)
4. Other entities

Responses to the information requested will assist the Government in determining the appropriate acquisition method, including whether any ISBEE/IEE or small business set-aside is possible based on the Contracting Officer's reasonable expectation of obtaining offers from two or more relevant concerns that will be competitive in terms of market price, product quality, and delivery capability. The applicable primary NAICS code for this requirement is 541611 – Administrative Management and General Management Consulting Services. **Generic capability statements are not sufficient and may not be considered when assessing entity capabilities. Please reference the Submission Information further below. Please also note the Key Personnel Requirements for this effort.**

Note:

- Respondents shall describe their ability to perform the attached Statement of Work within the anticipated period of performance (approximately September 2026 through July 2027). Respondents should clearly identify whether their capability assessment is based upon 1) adaptation of the existing BoostVax training framework or 2) development of a new CHW/CHR training program.
- If the respondent indicates the capability to adapt the existing BoostVax training framework, the response shall include the estimated transition period and identify any assumptions regarding the availability of existing training materials, source files, implementation documentation, evaluation tools, licenses, or other information that are material to the respondent's capability assessment.
- If the respondent indicates the capability to develop a new CHW/CHR training program from the ground up rather than adapting an existing program, the response shall describe the proposed approach, identify the major development activities, and estimate the period of time required to develop, pilot, evaluate, and disseminate the new training program, including any additional time beyond the anticipated period of performance that would be required to complete the effort. The respondent should also identify any assumptions regarding the availability of existing training materials or other information that are material to the proposed approach.

Capability Statement/Submission Information

A response to this SSN will be in the form of a capability statement to demonstrate capability and experience in the objectives identified further below. Specifically, please provide the following information below to demonstrate capabilities, including the capability to perform within the anticipated period of performance:

Cover Page

Please provide a cover page to include:

1. Company Name
2. Company Designation (ISBEE/IEE/SB/Other than Small Business)
3. Type of Business – as applicable (e.g. 8(a), Woman Owned Small Business, HUBZone, etc.)
4. Unique Entity Identifier (UEI) Number
5. Point of contact, email address, and phone number
6. Primary NAICS Code(s)
7. Existing federal contracting vehicle(s) numbers, as applicable (e.g. GSA MAS number, NITAAC/CIO SP3, OASIS, Alliant 2, or other Government Wide Agency Contracts, etc.)
8. System for Award Management (SAM) registration status. Note: all respondents must be registered on SAM (<https://sam.gov>) prior to any potential award.

Executive Summary

Provide a high-level overview of your services and include how it will benefit the Indian Health Service.

Technical Objectives

Respondents are expected to submit a capability statement detailing and demonstrating their expertise and technical capabilities to meet (as a prime contractor) the objectives outlined further below in this SSN, including but not limited to the Key Personnel requirements outlined below.

Past Performance

The Government is particularly interested in vendors with experience supporting similar federal public health. Please provide any past performance information that demonstrates your capabilities in the relevant technical objectives and magnitude of work. Please limit to no more than 5 relevant examples.

Representation

If identifying as an IEE/ISBEE, provide with your response to this notice a completed Indian Economic Enterprise Representation Form (attached).

Other Information (Optional)

You may include any other information that may be helpful in developing or finalizing the requirements of the potential acquisition.

Submission Format

To assist the Government in an expeditious review of your capabilities, the capability statement should be organized as follows:

- Cover page
- Executive Summary
- Technical Objectives
- Past Performance – limit to 5 examples
- Other Information, as applicable

The tailored capability statement should not exceed 10 pages. Font must not be less than 10-point and may be submitted in either Microsoft Word or Adobe Portable Document Format (PDF) using a standard 8-1/2 by 11 inch paper size. All proprietary information should be marked as such.

Generic capability statements are not sufficient and will not be considered compliant with the requirements of this notice.

Disclaimer and Important Notes

This notice does not obligate the Government to award a contract. Any information provided by industry to the Government as a result of this SSN is strictly voluntary. Responses will not be returned. No entitlements to payment of direct or indirect costs or charges to the Government will arise as a result of contractor submission of responses or the Government's use of such information. The Government reserves the right to use information provided by respondents for any purpose deemed necessary and legally appropriate. Any organization responding to this notice should ensure that its response is complete and sufficiently detailed to allow the Government to determine the organization's capabilities to perform the work. Respondents are advised that the Government is under no obligation to acknowledge receipt of the information received or provide feedback to respondents with respect to any information submitted. After a review of the responses received, a pre-solicitation synopsis and solicitation may be published on a Governmentwide Point of Entry (GPE). However, responses to this Notice will not be considered adequate responses to any resultant solicitation.

Confidentiality. No classified, confidential, or sensitive information should be included in your response. The Government reserves the right to use any non-proprietary technical information in any resultant solicitation(s).

Thank you for your interest in IHS programs.

BACKGROUND

The Indian Health Service (IHS) is a federal agency within the Department of Health and Human Services responsible for providing comprehensive health services to American Indians and Alaska Natives (AI/AN). As the principal federal health care provider and health advocate for AI/AN people, the IHS mission is to raise AI/AN health status to the highest possible level.

Ensuring high levels of immunization protection is critical in AI/AN communities, considering disparities in infectious disease. For example, influenza and pneumonia rates are nearly twice as high in AI/AN populations than in all-U.S. populations. Recognizing the importance of continually strengthening immunization programs, the IHS Health Promotion Disease Prevention and Public Health programs promote building the capacity of tribes and communities by increasing immunization confidence, awareness, and preparedness through the sharing of effective solutions and assisting health care professionals to promote immunization uptake.

As of 2022, fewer AI/AN children were protected against diseases such as diphtheria, influenza, pneumococcus, and rotavirus, when compared to non-Hispanic white children. In response, the IHS has developed policies (e.g., E3 Strategy) and public health approaches to increase effective health messaging and partnerships with tribal health organizations, academic institutions, state health departments, and tribal epidemiology centers. To address other service barriers, this effort will focus on solutions for community health worker education.

For healthcare providers, evidence-based communication techniques such as the presumptive approach, personalized messaging, and motivational interviewing (MI) have been shown to support providers in facilitating health behavior change around immunization decision-making. BoostVax is

a comprehensive health provider web-based training package that is community-driven and culturally grounded to increase immunization coverage. The BoostVax program will continue to encourage and train health care providers to engage with patients to improve the immunization rates of children and adults.

The Community Health Workers/Representatives (CHW/CHR) Program is a unique concept for providing health care, health promotion, and disease prevention services. CHWs/CHRs have demonstrated how they assist and connect with the community, and their work has become essential to the spectrum of Tribal community-oriented primary health care services. CHWs/CHRs are great advocates, in part because they come from the communities they serve and have tribal cultural competence. Their dedicated work has assisted many to meet their healthcare needs. The health promotion and disease prevention efforts that CHWs/CHRs provide have also helped people from the community improve and maintain their health. By providing health education, CHWs/CHRs have contributed to lowering mortality rates. As trusted front line public health workers within tribal communities, CHWs/CHRs can enhance immunization efforts through culturally tailored education, outreach, and clinic support.

PURPOSE/OBJECTIVE

The IHS proposes to increase the number of CHWs/CHRs who will support efforts to reduce the risk of infectious diseases in communities by encouraging the uptake of immunizations. To accomplish this, the contractor will tailor the existing BoostVax training package and program for healthcare providers to develop a CHW/CHR specific training module that is also community-driven, culturally grounded, and addresses CHW/CHR needs (including immunization education and communication, billing, and vaccine safety, CHW/CHR safety etc.).

SCOPE/TASKS TO BE PERFORMED

Community Health Worker (CHW) / Community Health Representative (CHR) Training

Objective 1: Design, Pilot, and Host the Training Package

- a. Compile best practices related to immunization education for Community Health Workers/Representatives (CHW/CHR), communication best practices, billing practices, and CHW/CHR safety while on the job.
- b. Design multimedia learning package (modules) for CHW/CHR on immunization communication with AI/AN community members, billing for services (for states that support billing for CHW/CHR work), and CHW/CHR safety.
- c. Produce visuals/job tools for CHW/CHR professionals, including slide decks, handouts, and communication tips (electronic and print versions).
- d. In consultation with IHS, partner with three IHS areas to pilot the training package, working with implementation champions representing various CHW/CHR professionals.
- e. Host and maintain online training package, including self-paced modules
- f. Maintain a record of planning activities, meeting discussions, and attendees and debrief IHS on health worker learnings
- g. The training must be developed specifically for CHW/CHR who serve the American Indians and Alaska Natives (AI/AN) population.

Objective 2: Evaluate the pilot CHW/CHR training package

- a. Administer pre- and post-training surveys to CHWs/CHRs who participated in training to measure how training affected provider knowledge, confidence, and practice.
- b. Report on pre- and post-training outcomes, lessons learned from implementation, training reach, CHW/CHR self-efficacy, and maintenance of communication strategies.

Objective 3: Disseminate training package within AI/AN-Serving CHW/CHR Networks

- a. Create a dissemination plan.
- b. Disseminate the training package into three to five additional IHS regions/areas beyond those areas included in the pilot program from Objective 2.
- c. Disseminate knowledge on immunization uptake strategies and evidence-based CHW/CHR practice in unique community contexts, including to Indigenous professional networks (e.g., AAIP, Indian Health Service, VacciNative).
- d. Organize and execute at least one 2-day in-person event that includes IHS Area Immunization Coordinators and CHR/CHWs to share the training package.
- e. Track the number of CHWs/CHRs that participate in the learning curriculum.

KEY PERSONNEL

The Senior Project Manager is designated as Key Personnel for this effort. A Senior Vaccine Consultant/Clinical Research Subject Matter Expert (SME) is also designated as Key Personnel for this effort. **The Senior Vaccine Consultant/Clinical Research SME shall be a physician with at least five years of experience as a vaccine subject matter expert with extensive knowledge and experience in immunology, clinical trials, safety surveillance, or regulatory pathways or hands-on experience in vaccine development, safety tracking, or public health communication.**

The names of Key Personnel, and the name(s) of any alternate(s) who shall act for the contractor in the absence of these Key Personnel, shall be provided to the Government as part of the contractor's proposal. If the Key Personnel are to be replaced, the contractor must notify the COR and Contracting Officer at least 30 calendar days in advance, submit written justification for the replacement, and provide the name and qualifications of any proposed substitute(s). All proposed substitutes shall possess qualifications equal to or superior to those of the Key Personnel being replaced, unless otherwise approved by the Contracting Officer. The contractor shall not replace Key Personnel without prior written approval from the Contracting Officer. A resume or CV for the proposed replacement shall accompany the Contractor's request for approval. If Key Personnel become unavailable due to circumstances beyond the Contractor's control, the Contractor shall promptly notify the Contracting Officer and COR and propose a qualified replacement for Government review and approval.

PLACE OF PERFORMANCE

Contract may be performed entirely off site at the contractor's facility or other. All work performed must take place within the United States.

TRAVEL

Travel-related performance tasks include site visits to AI/AN communities to conduct up to five workshops with CHR/CHWs to pilot and disseminate the learning curriculum. Locations will be selected based on community, CHW/CHR, contractor, and IHS staff availability. All travel shall be in accordance with Federal Travel Regulations and must be approved in advance by the Contracting Officer and COR before any costs are incurred.

DELIVERABLES/REPORTING SCHEDULE

Task	Deliverable	Format	Due Date/Frequency	Deliver To
Objective 1	Present to IHS compiled best practices	Electronic	10/5/26	COR & IHS POC
Objective 1	Present to IHS complete multimedia learning package (modules) and visuals/job tools (package continuing to be refined during pilot process)	Electronic	01/25/27	COR & IHS POC
Objective 1	Begin and continue pilot of multimedia learning package and visuals/job tools with three IHS areas	Electronic	02/01/27	COR & IHS POC
Objective 1	Continue to host and maintain online training package	Electronic	2/01/27 through end of contract period	
Objective 1	Debrief IHS on pilot progress	Electronic	03/01/27	COR & IHS POC
Objective 2	Complete evaluation of the pilot training package and visuals/job tools and report to IHS results of pre- and post-training outcomes	Electronic	05/03/27	COR & IHS POC
Objective 3	Present to IHS Completed Dissemination Plan and begin dissemination	Electronic	05/17/27	COR & IHS POC
Objective 3	Present to IHS interim report on dissemination status, including number of CHWs/CHRs that participate in the learning curriculum	Electronic	06/15/27 and 7/15/27	COR & IHS POC
Objective 3	Organize and Participate in in-person 2-day training event	In-person	05/01/27	COR & IHS POC

Task	Deliverable	Format	Due Date/Frequency	Deliver To
For all Objectives	Present to IHS report on general activities (with final overall project report presented 07/30/27)	Electronic	As needed and final report presented 7/30/27	COR & IHS POC

All deliverables will be accepted upon review and approval by project stakeholders, with final approval resting with the COR.

Note regarding deliverables:

The products and services provided under this contract shall have the following features and capabilities to ensure compatibility with the IHS CHR Learning Management System (LMS):

- A SCORM-compliant package (.zip file), SCORM 1.2 preferred
- Ready for Blackboard LMS
- Number of total hours in training
- No Flash, no outside links or LTI connection required
- Section 508 accessibility compliance
- Works on supported browsers (Chrome compatible and tested)

The Contractor shall deliver all final products in formats including but not limited to:

- Editable source files (e.g., PowerPoint, Word, design files)
- SCORM packages and source files for e-learning modules
- Video source files and caption files
- Documentation sufficient for future maintenance and updates